Centre universitaire de santé McGill | McGill University Health Centre

☐ HME ☒ HGM ☒ HRV
☐ HNM ☐ ITM ☐ CL

NP Rapid Access Cardiology Clinic Referral Form (Post-CCU Discharge)

No routine follow-up: refer only if there is a new or active clinical issue

➔ **Send to:** cardiotx@muhc.mcgill.ca and include **discharge summary**

Exclusions (Not for NP Rapid Access Clinic)

- ✖ Non-cardiac geriatric issues (falls, cognitive decline, polypharmacy unrelated to cardiac therapy)
- ✖ Stable chronic comorbidities (COPD, diabetes, arthritis, chronic renal insufficiency without acute cardiac trigger)
- ✖ General frailty without specific cardiac intervention needs (unless directly related to CHF optimization)
- ✖ Long-term follow-up for myocarditis/pericarditis (should be directed to subspecialty cardiology)
- ✖ Post-surgical patients where primary follow-up is surgical (e.g., valve replacement with surgeon)

Follow-Up Timeframe:

☐ **Within 1 week** (early discharge, high-risk clinical evolution) ☐ **7 – 10 days** (no acute issue anticipated)

Reason for Referral:

☐ Unstable coronary artery disease – follow-up after recent hospitalization or angiogram

☐ Structural heart patient (post TAVI / MR / TR) – close follow-up to detect conduction disturbances, pericardial effusion, clinical assessment residual/paravalvular leak, optimize medications (BP, renal, diuretics)

☐ Structural heart patient (post TAVI / MR / TR) – Same Day Discharge from Cath Lab – ***Support Patient Line***

☐ Post-ACS follow-up – re-evaluation after NSTEMI/STEMI, post-revascularization, or medical management

☐ Angina / Anti-anginal therapy adjustment – medication titration, symptom reassessment

☐ Arrhythmia management –example; atrial fibrillation/flutter, palpitations, post-cardioversion, rate/rhythm control

☐ Renal function monitoring – assess renal recovery post-contrast exposure, or after diuretic/GDMT reintroduction

☐ Pericardial disease – uncomplicated pericarditis, mild pericardial effusion *(non-surgical)*

☐ Myocarditis follow-up – subacute or mild cases requiring short-term reassessment

☐ Blood pressure concerns – orthostatic hypotension, uncontrolled hypertension, post-medication adjustment

☐ Post-pacemaker – Reassess medication, symptoms *(not pace clinic)*

☐ Anticoagulation monitoring – short-term INR follow-up after new initiation or dose change ***(long-term follow-up must already be arranged; RAC visit not for ongoing management)***

☐ Hematoma follow-up – post-catheterization access site evaluation

☐ Readmission prevention (frail / high-risk patients) – recently decompensated heart failure, advanced age, or multiple admissions within 3 months

☐ Other: _____

Tests Required before appointment:

☐ **Chem10** ☐ **CBC** ☐ **INR** ☐ **NT-proBNP** ☐ **ALT, AST** ☐ **CRP**

☐ **EKG** ☐ **Chest X-ray (CXR)** ☐ **Other:** _____

Comments / Additional Notes

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Referring Physician (Staff): _____ No permit: _____ Date: _____

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